Clinical Practice Guideline: Management of Type 2 Diabetes Mellitus in Primary Care (Abridged)

Guideline ID: DEMO-T2DM-2023-A **Version:** 1.0 (demonstration abridgement) **Effective Date:** May 2023 **Issuing Body:** Adapted for demonstration from the VA/DoD Clinical Practice Guideline for the Management of Type 2 Diabetes Mellitus, Version 6.0 (May 2023)

1. Scope and Purpose

1.1 Clinical Condition

This guideline covers glycemic targets, cardiorenal-protective therapy, and complication screening for adults with type 2 diabetes mellitus. It is a demonstration abridgement that keeps a small number of the source guideline's recommendations, with their original numbers, and omits the rest.

1.2 Target Population

Adults with type 2 diabetes mellitus managed in primary care.

1.3 Exclusions

Type 1 diabetes, gestational diabetes, and inpatient glycemic management are outside the scope of this abridgement.

2. Background

The source guideline sets glycemic targets as an individualized HbA1c range rather than a single number, and prioritizes drug classes with proven cardiovascular and renal benefit for patients with atherosclerotic cardiovascular disease, chronic kidney disease, or heart failure.

3. Recommendations

3.1 Glycemic Target (source recommendations 9, 10, 25)

An individualized HbA1c target range is set from the clinician's appraisal of risk and benefit, patient characteristics, complications, comorbidities, and life expectancy (Weak for). For most patients the range is 7.0 to 8.5 percent, if it can be safely achieved (Weak for). When glycemic control can be achieved with other treatments, drug classes other than insulin, sulfonylureas, or meglitinides are prioritized to minimize the risk of hypoglycemia, especially in patients aged 65 years and over (Weak for).

Decision Table 1: Glycemic Status

HbA1c (%)	Treated with Insulin or a Sulfonylurea	Glycemic Status	Action
< 7.0	Yes	Below range	Review therapy to reduce hypoglycemia risk
< 7.0	No	Below range	Continue current therapy
7.0 to 8.5	Any	Within range	Continue current therapy
> 8.5	Any	Above range	Intensify therapy, prioritizing classes other than insulin, sulfonylureas, or meglitinides

Key principles:

1 The target range 7.0 to 8.5 percent applies to most patients; an individualized range set by the clinician replaces it when documented.

1 "Above range" means the HbA1c value is greater than the upper bound of the range.

3.2 Cardiorenal-Protective Therapy (source recommendations 19, 20, 22)

For patients with atherosclerotic cardiovascular disease, a glucagon-like peptide-1 receptor agonist or a sodium-glucose cotransporter-2 inhibitor with proven cardiovascular benefit is recommended (Strong for). For patients at high risk of atherosclerotic cardiovascular disease, that is with chronic kidney disease, left ventricular hypertrophy, or heart failure, the same classes are suggested (Weak for). For patients with chronic kidney disease, a sodium-glucose cotransporter-2 inhibitor with proven renal protection is recommended (Strong for).

Decision Table 2: Protective Therapy Indication

Has Atherosclerotic Cardiovascular Disease	Has Chronic Kidney Disease	Has Heart Failure	Indicated Therapy
Yes	Any	Any	GLP-1 receptor agonist or SGLT2 inhibitor with proven cardiovascular benefit
No	Yes	Any	SGLT2 inhibitor with proven renal protection
No	No	Yes	GLP-1 receptor agonist or SGLT2 inhibitor with proven cardiovascular benefit
No	No	No	No protective agent indicated by this table

3.3 Complication Screening (source algorithm, sidebar on monitoring; recommendation 7)

All patients receive diabetes self-management education and support (Strong for). A comprehensive lower-extremity risk assessment including monofilament testing is performed annually. A retinal examination is obtained within 6 months of a new diagnosis and then every other year. The urine albumin-to-creatinine ratio is monitored at least annually.

3.4 Annual Screening Process

Every 12 months the care team queries the record for a lower-extremity risk assessment result within the preceding 12 months, a urine albumin-to-creatinine ratio result within the preceding 12 months, and a retinal examination result within the preceding 24 months. For each result that is not found, the care team requests a visit for that screening within 4 weeks and notifies the responsible clinician with routine urgency, naming the missing screening. These are direct record queries; no decision table applies to this process.

3.5 Glycemic Follow-up Process

This process applies when Table 1 yields "Above range" and therapy has been intensified; the clinician starts it at the therapy change, so the process start is the therapy change. The care team requests a follow-up visit, which may be delivered through telehealth (source recommendation 3, Weak for), at an interval from the process start that the clinician sets, and queries the record for a repeat HbA1c result within that same interval. If no repeat result is found when the interval ends, the care team notifies the responsible clinician with routine urgency. No decision table applies to this process beyond Table 1, and no other prerequisite is required.

Key principles:

1 Decision Tables 1 and 2 are the only decision logic in this guideline. The processes in 3.4 and 3.5 use record queries, visit requests, and notifications; they do not define additional decisions.

4. Evidence Summary

Recommendation	Source number	Strength
Telehealth delivery of care	3	Weak for
Diabetes self-management education and support	7	Strong for
Individualized HbA1c target range	9	Weak for
HbA1c range 7.0 to 8.5 percent for most patients	10	Weak for
GLP-1 RA or SGLT2 inhibitor with ASCVD	19	Strong for
GLP-1 RA or SGLT2 inhibitor at high ASCVD risk	20	Weak for
SGLT2 inhibitor with chronic kidney disease	22	Strong for
Prioritize classes with low hypoglycemia risk	25	Weak for

5. References

1 Department of Veterans Affairs, Department of Defense. VA/DoD Clinical Practice Guideline for the Management of Type 2 Diabetes Mellitus, Version 6.0. May 2023.